Clinical Nursing SOAP Note - Respiratory Imaging Review

Case ID	RNP-7A9C	**Patient**	55-year-old female
Care Area	Med-surg observation	**Reason**	Chest imaging reviewed after report of progressive respiratory symptoms
Sources	Radiology case packet and nursing handoff note	**Status**	Provider final treatment decision not documented in supplied materials

S - Subjective

Patient is documented as reporting progressive shortness of breath and cough.

No additional subjective symptom details are included in the supplied materials.

O - Objective

Imaging exam reviewed: Chest radiographs with right-side-down decubitus view. Finalized written radiology report is the authoritative interpretation for this case.

Imaging-supported findings from finalized report:

- Moderate-sized right pleural effusion which freely layers on the right-side-down decubitus view.
- Hazy opacification in the right lung base.
- No pneumothorax.
- Mild enlargement of the cardiopericardial silhouette.
- Median sternotomy changes.

Radiology report language: Moderate-sized right pleural effusion which freely layers on the right-side-down decubitus view. Hazy right basilar opacification is favored to represent a combination of layering pleural fluid and atelectasis.

Radiology impression: Simple pleural effusion.

No vital signs, oxygen saturation values, laboratory results, medication list, allergy list, or provider treatment orders are included in the supplied handoff materials.

A - Nursing Assessment Summary and Boundaries

Imaging-supported concern	Moderate-sized right pleural effusion, freely layering, with right basilar hazy opacity favored by radiology to reflect layering pleural fluid plus atelectasis. This finding can be clinically relevant to the reported shortness of breath and cough.
Important negative imaging finding	No pneumothorax was reported.
Not established by supplied materials	Cause of the effusion, current oxygenation status, bedside respiratory severity, infection status, cardiac contribution, medication needs, procedural need, or discharge plan.
Additional imaging findings needing context	Mild cardiopericardial silhouette enlargement and median sternotomy changes are reported, but no prior history or clinical interpretation is supplied in the available materials.

P - Nursing Plan / Provider Follow-up Needs

- Carry forward the finalized radiology impression of simple right pleural effusion and the absence of pneumothorax in nursing handoff.
- Monitor and document respiratory assessment findings within nursing scope and available orders; do not infer unsupported vital signs, oxygen values, laboratory results, or treatment response in this note.
- Notify/clarify with provider for final assessment and next steps, including clinical correlation with symptoms, physical exam, vital signs, oxygenation data, laboratory data if obtained, and cause of the effusion.
- Escalate per facility protocol for any acute worsening of breathing, chest pain, cyanosis, confusion, fainting, inability to speak full sentences, or other signs of respiratory distress.
- Use patient-facing teaching language that describes pleural effusion as fluid around the outside of the lung and explains that provider follow-up is needed before decisions about treatment are made.

Source-bounded documentation note: This SOAP note summarizes supplied imaging and handoff materials only. It is not a provider diagnosis beyond the radiology impression and does not document a treatment decision.